CardioVision AI

Multi-Vessel Risk Assessment & Clinical Vector Evaluation

RESEARCH PROTOTYPE EVALUATION REPORT — NOT FOR CLINICAL DIAGNOSIS

Evaluation Date: Tue, Oct 13, 2026, 09:17 PM
Patient ID: CV-NO-3D
Architecture: 57-Feature Four-Model Architecture
Active Mode: Standard Production Mode

PATIENT NAME	PATIENT ID	AGE / BIOLOGICAL SEX	HEIGHT / WEIGHT / BMI
No 3D Demo	CV-NO-3D	58 yrs · Male	170 cm / 72 kg (24.9 kg/m²)

1. Cardiovascular Prediction Summary

Multi-vessel predictive classification derived from the 57-dimensional clinical feature vector.

OVERALL CAD	LAD ARTERY	LCX ARTERY	RCA ARTERY
70.0%	**50.0%**	**30.0%**	**20.0%**
CAD	Stenotic	Stenotic	Stenotic
cad_model.pkl	lad_model.pkl	lcx_model.pkl	rca_model.pkl
Threshold: **50.0%**	Threshold: **50.0%**	Threshold: **50.0%**	Threshold: **50.0%**
High Est. Prob (≥70%)	Moderate Est. Prob (50–69%)	Low Est. Prob (<50%)	Low Est. Prob (<50%)

Model Estimation Notice: Values are model-estimated statistical classification probabilities, not clinically calibrated epidemiological risk scores or measured anatomical luminal stenosis narrowing percentages.

2. Clinical Input Measurements Summary

Patient clinical parameters from active evaluation session evaluated against informational reference benchmarks.

Clinical Parameter	Patient Value	General Reference Benchmark	Informational Evaluation Status
Resting Systolic BP (BP)	**130 mmHg**	< 120 mmHg (resting systolic)	Above reference (≥ 130 mmHg, Informational)
Resting Pulse Rate (PR)	**70 bpm**	60 – 100 bpm	Within standard resting range
Left Ventricular EF (EF-TTE)	**50%**	≥ 50%	LVEF ≥ 50% (Standard systolic function)
Fasting Blood Sugar (FBS)	**98 mg/dL**	70 – 99 mg/dL	Within fasting range (70–99 mg/dL)
Serum Creatinine (CR)	**1.0 mg/dL**	0.7 – 1.3 mg/dL	Within displayed adult reference range (Informational)
LDL Cholesterol	**100 mg/dL**	< 100 mg/dL	Above standard reference (≥ 100 mg/dL, Informational)
HDL Cholesterol	**39 mg/dL**	> 40 mg/dL (M) / > 50 mg/dL (F)	Below reference (< 40 mg/dL, Informational)
Triglycerides (TG)	**122 mg/dL**	< 150 mg/dL	Within reference (< 150 mg/dL)
Evaluated Clinical Vector	**57 Model Inputs**	Standardized 7-domain vector	34 numeric + 23 binary clinical indicators synchronized

3. Patient-Specific SHAP Explainability (CAD & LAD)

Feature contributions calculated via game-theoretic Shapley values. **Scale Note:** TreeExplainer outputs additive probability contributions (Δ probability). LinearExplainer outputs additive log-odds contributions (Δ log-odds). Scales are strictly separated.

CAD Model Explainability: AI Explainability Data Unavailable
SHAP explanation could not be computed (Engine offline). Model predictions remain valid.

LAD Artery Model Explainability: AI Explainability Data Unavailable
SHAP explanation could not be computed (Engine offline). Model predictions remain valid.

3. Patient-Specific SHAP Explainability (LCX & RCA)

Candidate and baseline branch vessel models with explicit scale separation and additivity verification.

LCX Artery Model Explainability: AI Explainability Data Unavailable
SHAP explanation could not be computed (Engine offline). Model predictions remain valid.

RCA Artery Model Explainability: AI Explainability Data Unavailable
SHAP explanation could not be computed (Engine offline). Model predictions remain valid.

4. 3D Coronary Heart Visualization

High-resolution anatomical perspective from the interactive Three.js 106-mesh BodyParts3D cardiac model.



5. Model Performance and Reliability

Standard Mode: Retrospective model evaluation metrics recorded during development on held-out test data.

Model	Active Model File	Target Vascular Outcome	Accuracy	F1 Score	ROC-AUC
CAD Model	cad_model.pkl	Overall Coronary Artery Disease	**85.2%**	89.7%	**0.917**
LAD Model	lad_model.pkl	Left Anterior Descending Stenosis	**65.6%**	67.7%	**0.756**
LCX Model	lcx_model.pkl	Left Circumflex Stenosis	**57.4%**	53.6%	0.584
RCA Model	rca_model.pkl	Right Coronary Artery Stenosis	**60.7%**	50.0%	0.652

Model Reliability Notice: Standard LCX and RCA baseline models exhibit limited discriminatory power in development cohort testing (ROC-AUC 0.584 and 0.652). Probabilities must be interpreted with extreme caution and never guide clinical interventions independently.

6. Research Limitations, Educational Guidance & Disclaimer

Scope of Research Evaluation & Clinical Considerations:
- Model estimates represent statistical pattern matching against retrospective cohorts, not definitive clinical diagnoses.
- Results must always be interpreted in full clinical context by a licensed medical practitioner.
- Machine learning probabilities cannot substitute for direct coronary angiography, CT angiography, or stress imaging.
- Diagnostic and management decisions depend on patient presentation, physical exam, and comprehensive clinical history.

RESEARCH PROTOTYPE — NOT A CERTIFIED MEDICAL DEVICE:
This software and report are intended strictly for educational, scientific research, and methodology evaluation purposes. They are not approved or cleared by any regulatory health agency (e.g., FDA, CE, PMDA) for clinical diagnostic or therapeutic decision-making. Predictions and SHAP explainability analyses do not substitute for formal cardiovascular imaging, laboratory testing, or physician consultation.